

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Ring Pessaries Local Enhanced Service (New and Replacement Ring Pessaries service)
Commissioner Lead	Paul Green, Director for Primary Care
Provider Lead	
Period	1 st April 2026 – 31 st March 2029 (3 months' notice will be required to terminate this contract by either party)
Date of Review	Annual review re inflation Formal LES review: September 2028 or sooner if required to align with commissioner priorities

1. Population Needs

1.1 National/local context and evidence base

Pelvic organ prolapse is a very common condition, with half of women over the age of 50 experiencing symptoms and 1 in 10 women by the age of 80 having surgery for prolapse.

Normally caused by pregnancy and childbirth, prolapse occurs when the pelvic floor muscles holding the organs within a woman's pelvis (uterus, bladder and rectum) are weakened or overstretched and the organs bulge from their natural position into the vagina.

Complications can start due to large birth weight babies, multiple childbirths, forceps or suction deliveries, episiotomies that are not repaired well. Women who have delivered via caesarean section are also susceptible to pelvic organ prolapse. Pelvic organ prolapse is not just restricted to women who have delivered. Those who have never delivered or who have never been pregnant also have been known to suffer with some kind of prolapse.

Other reasons for uterine prolapse may include:

- Ageing: uterine prolapse is more common in older women.
- Menopause: low levels of oestrogen can weaken tissues
- Being overweight: being heavy or obese can put a strain on the uterus
- Persistent cough: smoking or a lung condition can weaken muscles
- Long term constipation: straining to open the bowels can trigger prolapse
- Lifting or manual labour: repeated strain can weaken the pelvic tissues
- Genetic disorders: these can affect body tissues as in Marfan Syndrome
- Previous pelvic surgery: a hysterectomy or bladder repair may weaken the pelvic floor

Plain ring pessaries (not containing any hormonal or other substance) can be an effective symptomatic treatment for pelvic organ prolapse and are more acceptable to some patients than surgery.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	Preventing people from dying prematurely	
Domain 2	Enhancing quality of life for people with long-term	X

		<u>conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>		
	Domain 3	Helping people to recover from episodes of ill-health or following injury		
	Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	X	
	Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	X	

3. Scope

3.1 Aims and objectives of service

The aim of this service is to provide a service for the insertion, monitoring, and removal of ring pessaries for new and replacement ring pessaries according to current best practice. NICE guidelines state that a vaginal pessary should be considered for woman with symptomatic pelvic organ prolapse, alone or in conjunction with supervised pelvic floor training.

It aims to improve access to ring pessaries in primary care and reduce the need for patients to attend secondary care. It is intended to cover the enhanced aspects of clinical care of the patient, which are presumed to be beyond the scope of essential services and the quality and outcomes framework.

A ring pessary fitting service within a primary care setting will:

- Help reduce inequality of care across Devon through provision of an accessible and convenient services in an out of hospital environment
- Increase choice for all patients
- Provide an integrated and collaborative approach to ensure a seamless service
- Provide faster access to management of gynaecology conditions in primary care setting

In line with NICE guidelines, the pessary should be removed at least once every 6 months to prevent serious pessary complications. This is not classed as a minor surgical procedure, but practices are encouraged to follow minor surgery regulations and Infection Control guidelines

3.2 Service description/care pathway

The Provider is required to:

- **Produce and maintain a register:** of all patients fitted with a ring pessary to include:
 - Patient name and date of birth
 - Date of fitting
 - Type and size of pessary fitted

- Any special instructions / insertion details
- Replacement date
- Follow up arrangements
- Name of clinician who carried out procedure
- Read code – appropriately coded consultation

- **Call and Recall system:** Have in place a call and recall system to invite patients at appropriate intervals (usually every six months).
- **Counselling and Screening:** Provide appropriate counselling and screening prior to insertion in accordance with National Institute for Health and Care Excellence (NICE) Guidelines.
- **Fitting, monitoring, checking and removal** of ring pessaries as appropriate. As per NICE guidelines, more than 1 pessary fitting may be needed to find a suitable pessary.
- **Regular four to six monthly assessment:** where the pessary is removed, washed and reinserted and the vagina examined. Each pessary may be reinserted up to four times.
- **Patient consent:** In each case the patient should be fully informed of the treatment options and the treatment proposed. The patient should give verbal consent for the procedure to be carried out and this should be documented in the patient's lifelong medical record. This should be in line with GMC guidance for informed consent
http://www.gmc-uk.org/guidance/ethical_guidance/consent_guidance_index.asp
- **Re-referral to secondary care:** where the patients' symptoms or other complications require specialist advice;
- **Record keeping relating to the service:** including all known information relating to any significant events – i.e. deaths – of which the practice has been notified;
- **Staff Training:** Ensure primary care staff training – The provider must ensure that all staff involved in providing any aspect of care under this scheme have the necessary training and skills to do so. Practices should be able to demonstrate that they have in place a policy to cover staff training and maintenance of skills. The Devon Training Hub provide a 1-day course on pessary management.

[Pessary Management of Pelvic Organ Prolapse Primary Care Level | Devon Training Hub](#)

3.3 Population covered

The registered practice population.

3.4 Any acceptance and exclusion criteria and thresholds

Inclusions

All patients aged 18 and above, with symptomatic pelvic prolapse requiring vaginal ring pessaries in primary care.

Exclusions

An environment where there are inadequate or insufficient infection controls in place

3.5 Interdependence with other services/providers

The provider will be required to ensure they communicate as appropriate with other providers of Gynaecology services across Devon.

4. Applicable Service Standards

4.1 Applicable national standards (eg NICE)

The provider is required to adhere to all national standards as issued from time to time by any relevant Regulatory and Statutory bodies including guidance issued by appropriate competent bodies (eg Royal Colleges)

Urinary incontinence and pelvic organ prolapse in women: management NICE guideline [NG123]
Published date: April 2019 Last updated: June 2019 <https://www.nice.org.uk/guidance/ng123>

Please ensure that latest national infection control and prevention guidance is adhered to, along with appropriate personal protective equipment as risk assessed. Please follow current national guidance to include but not exclusive of:

<https://www.nice.org.uk/guidance/cg139>

<https://www.england.nhs.uk/coronavirus/primary-care/infection-control/>

<https://www.infectionpreventioncontrol.co.uk/gp-practices/https://ips.uk.net>

<https://thepogp.co.uk/professionals/resources/pessariesforpros.aspx>

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

The provider must have adequate mechanisms and facilities, including premises and equipment, as are necessary to enable the proper provision of this service.

The provider should report incidents according to their clinical governance policy

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

6. Location of Provider Premises

The provider premises.

7. Funding

New or Replacement Ring Pessary Fitting Price: £41.04 (25/26 rate)

Activity and funding will be reviewed annually.

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider